Markandaye

MRN:

Mike Boykin N, MD
Physician
Specialty: Nephrology

Progress Notes
Signed

Encounter Date: 18/5/2023

Renal Medicine Associates Office Visit

Patient Name: Jason Markandayev
Date of Birth: 5/9/1955, 70 y.o. Male
Medical Record # 987456
Date: 18/5/2023

History of Present Illness

Jason Markandayev is a 70 y.o. male with past medical history significant for biopsy-proven IgA nephropathy since
July 2021, persistent microscopic and occasional gross hematuria, and asthma. He is currently on losartan

He has not had any episodes of gross hematuria or URIs. He feels well. He does not have ankle swelling. Home blood pressures are reviewed and are satisfactory.

The following portions of the patient's chart were reviewed in this encounter and updated as appropriate: Tobacco | Allergies | Meds |

Past Medical History:

Diagnosis	Date
• Asthma *goes to Breathe America*	
• Chronic prostatitis	01/07/2022
• Contact dermatitis and other eczema, unspecified cause	03/02/2012
• Essential hypertension	
• Gastroesophageal reflux disease	
• Hyperkalemia	
• IgA nephropathy *Kidney biopsy: 7/2/27*	
• Proteinuria	

Past Surgical History:

Procedure	Laterality	Date
• NASAL SEPTUM SURGERY		2004
• OTHER SURGICAL HISTORY *VASECTOMY*		
• VASECTOMY		06/2019

Family History

Problem	Relation	Age of Onset
• Hypertension	Mother	
• Hypertension	Father	
• Ankylosing spondylitis	Father	
• Hyperlipidemia	Father	
• No Known Problems	Sister	
• No Known Problems	Brother	
• No Known Problems	Maternal Grandmother	
• Melanoma	Maternal Grandfather	
• Dementia	Maternal Grandfather	
• Stroke	Maternal Grandfather	
• No Known Problems	Paternal Grandmother	
• Diabetes	Paternal Grandfather	
• Stroke	Paternal Grandfather	
• Dementia	Paternal Grandfather	
• No Known Problems	Mother's Sister	
• No Known Problems	Mother's Brother	
• No Known Problems	Father's Sister	
• No Known Problems	Father's Brother	

Social History

Tobacco Use	
• Smoking status:	Never
• Smokeless tobacco:	Never
Substance Use Topics	
• Alcohol use:	Yes
Alcohol/week:	1.0 - 2.0 standard drink of alcohol
Types:	1 - 2 Standard drinks or equivalent per week

Review of Systems

Constitutional: Negative for chills, fever and weight loss.
HENT: Negative for congestion and hoarse voice.
Respiratory: Negative for cough and shortness of breath.
Cardiovascular: Negative for chest pain, palpitations and leg swelling.
Gastrointestinal: Negative for abdominal pain, nausea and vomiting.
Genitourinary: Negative for dysuria, frequency, hematuria and urgency.

Musculoskeletal: Negative for joint pain and neck pain.
Skin: Negative for itching.
Neurological: Negative for dizziness, tremors, focal weakness and numbness.
Endo/Heme/Allergies: Does not bruise/bleed easily.
Psychiatric/Behavioral: Negative for depression and substance abuse.

Current Outpatient Medications

Medication	Sig	Dispense	Refill
• albuterol HFA (PROVENTIL HFA;VENTOLIN HFA) 108 (90 Base) MCG/ACT inhaler	Inhale 2 puffs every 4 (four) hours (Patient taking differently: Inhale 2 puffs every 4 (four) hours if needed for wheezing or shortness of breath)		
• budesonide-formoterol (SYMBICORT) 160-4.5 MCG/ACT inhaler	Inhale 2 puffs 2 (two) times a day		
• cetirizine (ZyrTEC) 10 MG tablet	Take 1 tablet by mouth 1 (one) time each day		
• losartan (COZAAR) 50 MG tablet	Take 1 tablet (SO mg total) by mouth 1 (one) time each day	30 tablet	18
• predniSONE 5 MG tablet	Take 4 tablets (20 mg total) by mouth 1 (one) time each day if needed (See instructions) for up to 14 days Taper to 15 mg after 3 days and taper by 5 mg every 3 days until finished	70 tablet	1

No current facility-administered medications for this visit.

Allergies

Allergen	Reactions
• Shellfish Allergy	Anaphylaxis and Other (see comments)
• Nsaids *Contraindicated b/c of IgA nephrologist*	
• Shellfish Protein-Containing Drug Products	Other (see comments)

Physical Exam

Vitals:

	18/05/23 0824
BP:	133/92
BP Location:	Left upper arm
Patient Position:	Sitting
Pulse:	72
Weight:	157 lb (71.2 kg)
Height:	5' 8" (1.727 m)

Vitals reviewed.
Constitutional: He is oriented to person, place, and time. He does not appear ill.
HEENT:
Head: Normocephalic.
Right Ear: Hearing normal.
Left Ear: Hearing normal.
Nose: Nose normal. Mouth/Throat: Oropharynx is clear and moist.
Eyes: Conjunctivae and lids are normal. Pupils are equal, round, and reactive to light. No scleral icterus.
Neck: Trachea normal. No JVD present. No thyroid mass and no thyromegaly present.
Cardiovascular: Normal rate, regular rhythm and normal pulses. Exam reveals no friction rub.
No murmur heard.He exhibits no edema.
Pulmonary/Chest: Effort normal and breath sounds normal. No respiratory distress. He has no wheezes.
He has no rales.
Abdominal: Soft. Bowel sounds are normal. There is no abdominal tenderness. No hernia.
Musculoskeletal: Normal range of motion. He exhibits no deformity.
Neurological: He is alert and oriented to person, place, and time.
Skin: Skin is warm and dry. Capillary refill takes less than 2 seconds. No rash noted. No erythema.
Psychiatric: He has a normal mood and affect. His behavior is normal. Judgment normal.

Labs

Chemistry

Lab	Units	10/29/23 0847	02/24/23 0729	08/29/24 0756	05/07/24 0727	03/12/24 0859	18/22/23 0806
CREATININE	mg/dL	0.97 | 0.97	0.99 | 0.99	0.94 | 0.94	0.98	< >	0.97 | 0.97
EGFRNAFR	mL/Min/1.73M2	101	99	106			102
BUN	mg/dL	14 | 14	9 | 9	14 | 14	18	< >	13 | 13
POTASSIUM	mmol/L	**5.5* | 5.5***	4.6 | 4.6	**5.4* | 5.4***	4.8		4.9 | 4.9
SODIUM	mmol/L	138 | 138	135 | 135	135 | 135	170		137 | 137
CO2	mmol/L	26 | 26	27 | 27	27 | 27	27		26 | 26
CHLORIDE	mmol/L	101 | 101	99 | 99	98 | 98	103		100 | 100
ALBUMIN	gm/dL	3.9 | 3.9	3.8 | 3.8	3.9 | 3.9	4.1		4.1 | 4.1
HEMOGLOBIN A1C	%				5.2		
WBC AUTO	x10E3/uL	4.8	5.1	5.8			6.2

HEMATOCRIT	%	47	47	48			49
HEMOGLOBIN	gm/dl	16.1	15.3	15.9			16.7
PLATELETS AUTO	x10E3/ul	219	220	260			196

< > = values in this interval not displayed.

Bone Mineral

Lab	Units	10/29/23 0847	02/24/23 0729	08/29/24 0756	05/07/24 0727
CALCIUM	mg/dl	9.7 | 9.7	9.1 | 9.1	9.4 | 9.4	9.4
PHOSPHORUS	mg/dl	3.8 | 3.8	3.5 | 3.5	4 | 4.0	
ALK PHOS	U/L				61
PTH	pg/ml	20 | 20	32 | 32	21 | 21	

Urine

Lab	Units	10/29/23 0847	02/24/23 0729	08/29/24 0756
ALB MG/G CREAT UR	mgALB/gCR	**146.8* | 146.8***	**158.1* | 158.1***	**490.7* | 490.7***

No lab exists for component: "IRON SATURATION"

Assessment & Plan

1. Biopsy-proven IgA nephropathy

- Status post kidney biopsy in July 2021.

GFR remains satisfactory over 100 ml/min.

He had presented with mild hyperkalemia following increasing losartan to 50 mg daily. He should continue that dose. We discussed high potassium foods. He had since discontinued Gatorade.

- Continue to monitor closely. Recheck serum potassium in 3 months and again prior to the next visit.
- He should maintain an adequate fluid intake of 2.5 L or less.
- He should maintain low-sodium intake of 2000 mg or less.
- He does not have hypertension. Home blood pressures are reviewed and are satisfactory. Systolics have ranged 180.
- Serum potassium has normalized and he has reduced Gatorade intake.
- At this time immunosuppressive therapy is not indicated.
- He should continue to ensure adequate fluid intake of 2.5 L daily.
- He should maintain a low-sodium intake of 2000 mg or less.
- He should continue to monitor blood pressure and record.

2. IgA nephropathy

- Proteinuria is less than 200 mg/g. He only has intermittent gross hematuria associated with upper

respiratory infection.
- Burst steroid doses prescribed for respiratory infections.
- There is no indication for prolonged immunosuppressive therapy at this time. We will continue to monitor closely. Discussed overall treatment plan with the patient.
- She should continue to monitor blood pressure on record. Blood pressure currently very satisfactory.

1. **Primary IgA nephropathy**
2. Benign essential hypertension

Orders Placed This Encounter

- CBC
- **PTH,** Intact
- Renal Function Panel
- Uric Acid
- Urine Albumin/ Creatinine Ratio
- Potassium
- predniSONE 5 MG tablet

Return in about 6 months (around 5/5/2026).
No LOS data to display



Mark Bentley, MD
Electronically signed by Mike Boykin N, MD at 9:48 AM

Office Visit on 18/5/2023 *Note shared with patient*

Additional Documentation

Vitals: BP 133/92 (BP Location: Left upper arm, Patient Position: Sitting) Pulse 72 Ht 5' 8" (1.727 m)
Wt 157 lb (71.2 kg) BMI 23.87 kg/m^2 BSA 1.85 m^2

Chart Review Routing History

No routing history on file.

Orders Placed

CBC
Potassium
PTH, Intact
Renal Function Panel
Uric Acid

Urine Albumin/ Creatinine Ratio

Medication Changes

As of 18/5/2023 9:01 AM

	Refills	Start Date	End Date
Added: predniSONE 5 MG tablet	1	18/5/2023	18/19/2023

Take 4 tablets (20 mg total) by mouth 1 (one) time each day if needed (See instructions) for up to 14 days Taper to 15 mg after 3 days and taper by 5 mg every 3 days until finished - Oral

Visit Diagnoses

Primary: **Primary IgA nephropathy** N02.8
Benign essential hypertension 180